PROFESSIONAL TIP · ETHICS

Ethical Considerations When Choosing Treatment Modalities

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> **Disclaimer:** This tip contains my personal opinions on ethical topics. It is not legal advice. Read the full disclaimer.

In my opinion, counselors are ethically required to use the most effective and scientifically validated treatment modalities to ensure the highest standard of care. This obligation is outlined in the ethical codes of professional organizations like the American Counseling Association (ACA) and the American Psychological Association (APA), which emphasize evidence-based practices to protect client welfare and prevent harm.

Below are the ethical principles and guidelines that inform counselors' responsibility to use appropriate treatment methods.

1. Beneficence and Nonmaleficence (Do No Harm)

Counselors are ethically obligated to act in the best interest of their clients by selecting treatments supported by scientific research. **Beneficence** means promoting the well-being of the client by providing interventions that benefit them, improve their condition, and enhance their quality of life. **Nonmaleficence**, often referred to as "Do No Harm," is the obligation to avoid causing harm to the client through harmful or ineffective interventions.

These principles are clearly outlined in the ethical codes:

- **ACA Code of Ethics (2014)**, **Section A.4.a** (Avoiding Harm): "Counselors act to avoid harming their clients, trainees, and research participants and to minimize or to remedy unavoidable or unanticipated harm."
- **APA Ethical Principles of Psychologists and Code of Conduct (2017)**, **Principle A** (Beneficence and Nonmaleficence): "Psychologists strive to benefit those with whom they

work and take care to do no harm." (APA describes its General Principles as aspirational; the enforceable rules are in its Ethical Standards.)

One type of harm that clients may experience is financial harm: paying for treatments that did not work, or that took extended periods of time and money to show results, when more effective and shorter-term treatments were available.

Both codes name nonmaleficence directly. The ACA Code lists it in its **Preamble** as one of the fundamental principles of professional ethical behavior, and the APA's Principle A is titled "Beneficence and Nonmaleficence." Any intentional wrongdoing or unethical action that harms a client is a clear violation of this principle. It also conflicts with **Section A.1.a** (Primary Responsibility) of the ACA Code, which states that "the primary responsibility of counselors is to respect the dignity and promote the welfare of clients."

Implications of nonmaleficence: Counselors must ensure that interventions do not cause harm, worsen a client's condition, or delay appropriate care. Harmful or unethical practices are strictly prohibited, and counselors must always aim to avoid causing harm.

2. Evidence-Based Practice (EBP)

Evidence-based practice involves integrating the best available research with clinical expertise and client preferences. Counselors are ethically required to select scientifically supported treatments when developing intervention plans.

The **ACA Code of Ethics (2014)**, **Section C.7.a** (Scientific Basis for Treatment), states: "When providing services, counselors use techniques/procedures/modalities that are grounded in theory and/or have an empirical or scientific foundation." Similarly, the **APA Ethics Code (2017)**, **Standard 2.04** (Bases for Scientific and Professional Judgments), states that psychologists' work "is based upon established scientific and professional knowledge of the discipline."

3. Competence

Counselors must practice within the boundaries of their competence, meaning they should only use treatments they are trained to deliver and that are supported by research. Competence

involves both knowledge and the ability to apply the most effective, evidence-based interventions.

The **ACA Code of Ethics (2014)**, **Section C.2.a** (Boundaries of Competence), requires counselors to practice only within the boundaries of their competence. If a counselor is unfamiliar with an evidence-based treatment for a client's condition, the counselor should seek training, supervision, or consultation (see **Section C.2.b**, New Specialty Areas of Practice) or refer the client to a more qualified provider (see **Section A.11.a**, Competence Within Termination and Referral). Similarly, the **APA Ethics Code (2017)**, **Standard 2.01** (Boundaries of Competence), requires psychologists to provide services only within the boundaries of their competence.

4. Informed Consent and Client Autonomy

Counselors are responsible for recommending evidence-based treatments, but clients have the right to make informed decisions about their care. Counselors must provide clients with clear information about the benefits, risks, and alternatives of various treatment options, including the level of scientific support behind those options.

The **ACA Code of Ethics (2014)**, **Section A.2.b** (Types of Information Needed), requires counselors to explain to clients the nature of all services provided, so that clients can make informed decisions about their treatment. Similarly, the **APA Ethics Code (2017)**, **Standards 3.10** (Informed Consent) and **10.01** (Informed Consent to Therapy), require psychologists to inform clients, as early as is feasible, about the nature and anticipated course of therapy and to give them the chance to ask questions and receive answers.

5. Professional Development and Continuous Learning

Counselors are ethically obligated to engage in ongoing professional development to stay current with the latest research and treatment modalities. This includes continuing education to ensure they are using the most effective, evidence-based interventions available.

The **ACA Code of Ethics (2014)**, **Section C.2.f** (Continuing Education), requires counselors to keep a reasonable level of awareness of current scientific and professional information and to

keep up to date with best practices. Similarly, the **APA Ethics Code (2017)**, **Standard 2.03** (Maintaining Competence), states that psychologists "undertake ongoing efforts to develop and maintain their competence."

6. Avoiding Harm from Unproven Treatments

Counselors must avoid using interventions that are not supported by research or that have been discredited, as these may result in harm or worsen a client's condition. When validated options are available, using unproven methods may violate the principle of **nonmaleficence**, as it risks causing unnecessary harm.

The **ACA Code of Ethics (2014)**, **Section C.7.c** (Harmful Practices), states that counselors do not use techniques, procedures, or modalities when substantial evidence suggests harm, even if such services are requested. This works together with **Section A.4.a** (Avoiding Harm) and **Section C.7.a** (Scientific Basis for Treatment). Similarly, the **APA Ethics Code (2017)**, **Principle A** (Beneficence and Nonmaleficence), stresses the importance of interventions that promote client well-being and avoid harm.

7. Cultural Sensitivity and Client Preferences

While evidence-based treatments are a priority, counselors must also consider the cultural background and individual preferences of clients. This may involve adapting evidence-based treatments to fit the client's cultural context, while ensuring that the treatment remains effective.

The **ACA Code of Ethics (2014)**, **Section A.2.c** (Developmental and Cultural Sensitivity), requires counselors to communicate information in ways that are both developmentally and culturally appropriate. The **APA Ethics Code (2017)**, **Standard 2.01(b)** (Boundaries of Competence), also emphasizes the knowledge and skills needed to serve people of different cultural backgrounds effectively.

8. Emerging Treatments and Ethical Flexibility

In some cases, established evidence-based treatments may not be available for specific conditions. In such instances, counselors must use their professional judgment, ethical reasoning, and the best available research to select the most appropriate treatment, while clearly communicating any limitations to the client.

The **ACA Code of Ethics (2014)**, **Section C.7.b** (Development and Innovation), allows the use of developing or innovative techniques when counselors explain the potential risks, benefits, and ethical considerations, and work to minimize any potential harm. Similarly, the **APA Ethics Code (2017)**, **Standard 10.01(b)** (Informed Consent to Therapy), requires psychologists who use a treatment for which generally recognized techniques have not been established to tell clients about the developing nature of the treatment, its potential risks, the alternative treatments that may be available, and the voluntary nature of their participation.

9. Use of the Best Available Treatment vs. Second-Best or Outdated Treatments

In my opinion, when a counselor has a choice between using the best, most efficacious, evidence-based treatment or a second-best or outdated treatment, it is ethically imperative to choose the most scientifically validated option. This decision aligns with the principles of **beneficence** and **nonmaleficence**, which require counselors to promote client well-being and avoid harm.

While there may be circumstances in which a second-best treatment is considered (such as client preferences, cultural sensitivity, or access to care), it is essential that the counselor makes clients aware of the evidence supporting each option. The **ACA Code of Ethics (2014)**, **Section C.7.a**, requires that counselors use techniques that are grounded in theory and/or have an empirical or scientific foundation. Using a less effective or outdated treatment without justifiable reasons may violate the ethical mandate to provide the best care possible.

Counselors must explain the risks, benefits, and efficacy of each option to the client as part of the informed consent process. This ensures that clients understand the limitations of second-best or outdated treatments and can make fully informed decisions about their care. However, even in

cases where the client prefers a second-best treatment, the counselor has an ethical obligation to ensure that the chosen intervention still meets a reasonable standard of care and is not harmful.

Conclusion: Ethical Responsibility to Uphold Nonmaleficence and Use Validated Treatments

Counselors are ethically obligated to use scientifically validated and effective treatment modalities. While evidence-based practices should be prioritized, they must be adapted to align with client preferences and cultural sensitivity. The principle of **nonmaleficence** (Do No Harm) and the avoidance of intentional wrongdoing are central to counseling ethics. Counselors must ensure that they do not cause harm to clients, either by using ineffective practices or treatments or through intentional misconduct. When evidence-based treatments are unavailable, counselors must inform clients of the limitations and monitor progress closely to ensure their well-being. Furthermore, when choosing between the best and second-best or outdated treatments, it is imperative to prioritize the most scientifically supported option, ensuring the best possible care for clients.

References

American Counseling Association. (2014). *ACA code of ethics*. https://www.counseling.org/docs/default-source/default-document-library/ethics/2014-aca-code-of-ethics.pdf

American Psychological Association. (2017). *Ethical principles of psychologists and code of conduct* (2002, amended effective June 1, 2010, and January 1, 2017). https://www.apa.org/ethics/code

Note: The ACA is revising its 2014 Code of Ethics. Until a new edition is adopted, the 2014 Code remains in effect. Section numbers may change in the new edition.

Disclaimer

This Professional Tip contains my personal opinions regarding ethical topics in counseling. Interpretations of ethics codes may differ among professionals, and ethics codes, laws, and licensing rules change over time and vary from state to state.

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